CASE SCENARIO :

Patient name : S A

Age: 72 year

Gender: Male

Allergy status : No known allergy

Past Medical history : Hypertension, Type 2 Diabetic mellitus, Old CVA

Present illness : Recurrent ais - right mca-pca watershed infarct + right parietal infarct with left hemiplegia, old right ica distal occlusion (only on tab stiloz),dvt left ll, ?sepsis with septic shock, ckd, ild - usual interstitial pneumonia, type 2 dm (hba1c: 7.7%),systemic hypertension, hfref (ef: 34%), Anemia of chronic disease, old pulmonary tb (30 years back)

Past medication history : Tab Stiloz 50 1-0-1
Tab Atorva 0-0-1
Tab Amlodac AT (Amlodipine & Atenolol)
Tab Dytor 10 1-0-0
Tab Cospiaq Sm (Empa+Sita+MF) 25/100/100mg 1/2-0-1/2

A 72 Year old male patient was Started on 18/9/2026 Tab Bisobis (Bisoprolol) 2.5mg For HFrEF and Hypertension. Later on 19/9/2026 he developed Hypotension. The drug was stopped. On 20/09/2026 Blood pressure came to normal 130/76mmhg.

Laboratory data :

Physical examination: on 19/9/2026 Blood pressure: 90/60 mmhg ,

20/9/2026 Blood presuure:130/76mmhg

Management:

Tab Bisobis 2.5 mg was discontinued due to hypotension. Noradrenaline support was documented for hypotension. Blood pressure monitoring was advised every 2 hours.

TASK 1: ADVERSE DRUG REACTION IDENTIFICATION:

Suspected reaction: Hypotension

Suspected drug : Tab Bisobis 2.5mg (Bisoprolol) once daily

Explanation : Bisobis contains bisoprolol, a selective beta-1 adrenergic receptor blocker. It blocks beta-1 receptors in the heart, thereby reducing heart rate and myocardial contractility, which can lower blood pressure. In this patient, the blood pressure was recorded as 98/70 mmHg and subsequently 90/66 mmHg. The patient was also receiving noradrenaline support for hypotension. Therefore, Tab. Bisobis was stopped in view of hypotension

So I decided that the suspected drug is Tab bisobis(Bisoprolol) caused Hypotension.

ADR REPORT FORM OF THIS CASE SCENARIO IS ATTACHED HERE

Version 1.4

SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) :1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☑ | Follow-up Case ☐

FOR AMC / NCC USE ONLY

Reg. No. / IPD No. / OPD No. / CR No. : 3300 21482

AMC Report No. :

Worldwide Unique No. :

A. PATIENT INFORMATION *

1. Patient Initials: SA
2. Age or date of birth: 72Y
3. Gender: M ☑ F ☐ Other ☐
4. Weight (in Kg.)

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy): 19/9/26
6. Event / Reaction stop date (dd/mm/yyyy): 20/9/26
7. Describe Event/Reaction management with details , if any

A 72Y old Male patient was on Tab Bisobisv 2.5mg for the treatment for HFrEF, & Hypertension. Later on 19/9/26 JT was stopped. Patients Developed Hypotension on 19/9/26 - BP - 90/60mmhg.

12. Relevant investigations with dates :

19/9/26 BP - 90/60mmhg
20/9/26 BP - 130/76mmhg

13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)

K/c/o T2DM, HTN, Anemia, Old CVA.

14. Seriousness of the reaction : No ☑ If Yes ☐ (*please tick anyone*)

☐Death (dd/mm/yyyy) ☐Congenital-anomaly
☐Life threatening ☐Disability
☐Hospitalization-Initial/Prolonged ☐Other Medically Important

15. Outcome:

☑Recovered ☐Recovering ☐Not Recovered
☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
i	T Bisobis (Bisoprolol)				2.5mg	PO	1-0-0	19/9/26	19/9/26	HFrEF & HTN	Probable
ii											Possible
iii											
iv#											

9. Action taken after reaction (*please tick*)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown
i	✓					
ii						
iii						
iv						

10. Reaction reappeared after reintroduction of suspected medication (*please tick*)

Yes	No	Effect unknown	Dose (if re-introduced)

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
i	T Stilo2 (Cilostazol)	50mg	PO	1-0-1	19/9/26		To Prevent Ischemic Stroke
ii							
iii#							

Additional Information :

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D. REPORTER DETAILS *

16. Name & Address : Shweta . XXX

Pin 575002 Email : Shwetakotian72@gmail.com

Contact No- :

Occupation : Pharm D Intern Signature :

17. Date of this report (dd/mm/yyyy) :

Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form